



Armada Kalamunda Group

Podiatry Referral Triage Guideline

1. Purpose

To outline Armada Health Service Podiatry eligibility criteria and prioritisation guideline for triaging of referrals. This will ensure that all Podiatry staff that perform triage, will triage consistently.

2. Applicability

Eligibility criteria for patients referred to Armada Health Service Podiatry include:

- High risk feet, characterised by the presence comorbidities such as Diabetes, Rheumatoid Arthritis, Chronic Renal Disease, Peripheral Vascular Disease, Peripheral Neuropathy, Inflammatory Joint Disease, or Immunocompromised patients.
- Patients must fall within the AKG catchment.
- Referrals must include a full medical history and history of the presenting complaint

3. Guidelines

3.1 Prioritisation framework:

Priority	Patient presentation
Emergency or Inpatient Care Indicated	Typically, these patients require immediate care due to risk of a limb threatening situation and/or death. Please refer patients to their nearest Emergency Department. The patient's condition is likely to improve with podiatric input or may assist in verifying a patient warrants this level of care. Patients typically present with (but not limited to): • Severe foot infection with systemic features • Chronic Limb Threatening Ischaemia /Acute Limb ischemia • Rapidly progressing/deteriorating condition e.g., necrotising infection, extensive gangrene • Severe Peripheral Arterial Disease with a foot ulcer • Foot ulceration with moderate infection associated with key morbidities • Multidisciplinary Foot Ulcer Clinic Referral indicated.

	Should a current patient or new referral indicate the above features, appropriate escalation to the nearest Emergency Department should be recommended as soon as identified.
Category 1 Urgent Within 0-30 days	Typically, these patient's require urgent Podiatry care due to the presence of a foot ulcer with or without infection. The patient's condition is likely to improve with podiatric input, it may prevent admission and may reduce the risk of deterioration. Generally, if care is not received there is serious risk to the patient's long-term health e.g., amputation or permanent disability. • Acute Charcot Neuroarthropathy • Foot ulcer • Peripheral Arterial Disease with foot ulcer. • Foot ulceration probing to deeper structures • Foot ulceration with mild infection. • Foreign body in the foot of a patient that is high-risk of foot disease. • Infected ingrown toenail in a patient that is high-risk of foot disease. Consider Multidisciplinary Foot Ulcer Clinic Referral.
Category 2 Routine Within 31-90 days	Typically, these are patients that have high-risk foot disease and have had previous active foot disease concerns. While they are generally considered stable, they have significant risk of reoccurrence of active concerns and management of reduction of risk factors in a timely manner will reduce the risks of reoccurrence. This includes a patient with Loss of Protective Sensation or Peripheral Arterial Disease and one or more of the following: • History of a foot ulcer • A lower-extremity amputation (minor or major) • End-stage renal disease • Stable/Chronic Charcot Neuroarthropathy.
Category 3 Non-Urgent Within 91-365 days	Typically, these patients are at a moderate risk of foot disease. They do not usually require long term care rather benefits can be seen with improved education, improved function/mobility, or improved falls risk factors.

	A person at moderate risk of foot disease has: • Loss of Protective Sensation + Peripheral Arterial Disease • Loss of Protective Sensation + Foot Deformity • Peripheral Arterial Disease + Foot Deformity This may include people who are CAEP eligible.
Community Care Recommended	Where foot health can be maintained sufficiently in the community with Medicare care plan services or services provided via alternative funding (e.g. NDIS, Moorditj Djena, nursing home, aged care package, private health and DVA patients among others). • High risk patients with feet that can be maintained sufficiently with community care. • Moderate risk ongoing care • Low and very-low risk • No comorbidities/risk factors to optimal foot health. • Paediatric patients

Referrals may be rejected if:

- They do not contain sufficient medical history and indication for review.
- Live outside AKG catchment
- Do not meet eligibility criteria

4. Roles and responsibilities

Daily review of eReferrals, both inpatients and outpatients, is the responsibility of all Podiatrists at AHS. Triage of outpatient referrals is the role of the most senior podiatrist on duty that day.

If any podiatrist is notified of an immediate referral (requiring review within 7 days) via phone, notify the referrer of acceptance and request the referral faxed/emailed to ensure timely review. You can discuss with your senior or transfer the phone call to your senior if you are unsure how to direct the referrer.

5. Supporting information

The following documents support the implementation of this policy:

- Outpatient Referral Triage and Management Guidelines.

6. Compliance, monitoring and evaluation

Compliance against this guideline will be evaluated using power BI dashboard for auditing of time to triage and time to review.

7. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 5: Action 5.11

8. References

1. Diabetes Feet Australia. (2021). 2021 Australian Guideline on prevention of Foot Ulceration (Version 1.0 140921). [New Guidelines - Diabetes Feet Australia](#)
2. International Working Group on the Diabetic Foot. (2023). IWGDF 2023 Guidelines. [All IWGDF Guidelines \(2023 update\) - IWGDF Guidelines](#)

9. Glossary

The following definitions are relevant to this policy.

Term	Definition
High risk	A person with Loss of Protective Sensation or Peripheral Arterial Disease and one or more of the following: • History of a foot ulcer • A lower-extremity amputation (minor or major) • End-stage renal disease
Moderate risk	A person with: • Loss of Protective Sensation + Peripheral Arterial Disease • Loss of Protective Sensation + Foot Deformity • Peripheral Arterial Disease + Foot Deformity
Low risk	A person with: • Loss of protective sensation or Peripheral Arterial Disease
Very-Low risk	A person without loss of protective sensation or Peripheral Arterial Disease.

10. Document owner

Podiatry Coordinator

Enquiries relating to this guideline may be directed to:

Title: Podiatry Coordinator
Department: Podiatry Department
Email: AHS.Podiatry@health.wa.gov.au

11. Review

This guideline will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V2.0	25/06/2025	25/06/2028	Section 3.1 – Updated terms to reflect best evidence Added associated with key morbidities Removed terms not needing immediate ED/IP Care

12. Authorisation

This guideline has been authorised and issued by the Director Allied Health and Ambulatory Care.

Authorised by	Danielle Kilmurray – Director Allied Health and Ambulatory Care
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